

Pennsylvania 2027 Health Insurance Rate Filings and Market Dynamics: A Public Health White Paper on Policy, Affordability, and Coverage Retention

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Abstract

This white paper examines proposed Plan Year 2027 health insurance rate filings submitted to the Pennsylvania Insurance Department (PID), evaluating market dynamics, enrollment attrition, and implications for population health equity. For 2027, Pennsylvania health insurers requested enrollment-weighted average premium increases of 17.1% in the individual market and 11.5% in the small-group market. Individual market rate change requests ranged from 10.46% (Geisinger Health Plan) to 40.90% (Ambetter Health of Pennsylvania). A critical driver of Ambetter's request was a bronze-tier exit resulting in a 17.46% bronze-to-silver plan-mapping differential layered onto a 23.44% silver-to-silver rate hike. Concurrently, longitudinal data from Pennie (Pennsylvania's state-based marketplace) demonstrated a 13.0% decline in active enrollment between February 1 and August 1, 2026 (falling from 486,000 to 422,869 enrollees). This attrition followed the end-of-2025 expiration of enhanced federal premium tax credits, which doubled net monthly premium burdens for average enrollees. Following federal litigation (*City of Columbus v. Kennedy*), Pennie restored the traditional 15-week open enrollment period (November 1, 2026–January 15, 2027). This paper highlights the public health risks of premium shock, coverage churn, and tier migration, offering policy recommendations to safeguard health equity and continuity of care.

Keywords: Health insurance rate filings, Pennie, ACA marketplace, premium tax credits, health equity, coverage churn, Pennsylvania public health.

Executive Summary and Policy Context

In July 2026, the Pennsylvania Insurance Department (PID) released proposed rate filings for Plan Year 2027. Insurers operating across the state requested an enrollment-weighted average premium increase of 17.1% in the individual market and 11.5% in the small-group market (Pennsylvania Insurance Department [PID], 2026). These proposed rates are subject to administrative review, public comment, and formal approval by the Insurance Commissioner before finalization in autumn 2026.

The proposed increases coincide with severe post-enrollment coverage attrition across Pennsylvania's state-based exchange, Pennie. Following the expiration of enhanced federal premium tax credits on December 31, 2025, individual marketplace enrollees experienced a mean net premium payment increase of approximately 102% (WVIA, 2026). Active Pennie enrollment subsequently contracted by 63,131 individuals between February 1 and August 1,

2026, signaling diminished premium affordability among working-class and moderate-income households.

Simultaneously, legal challenges regarding federal open enrollment duration limits culminated in a judicial stay (*City of Columbus v. Kennedy*, 2026), enabling Pennie to restore its 15-week open enrollment window (November 1, 2026–January 15, 2027) rather than enforcing a shortened 9-week period (CMS, 2026; Pennsylvania Office of Rural Health [PORH], 2026). This white paper contextualizes these statutory, financial, and market dynamics through a public health framework.

2027 Individual Market Rate Requests and Structural Mechanics

Individual market rate requests reflect substantial variance across geographic rating areas and insurer product structures. Table 1 outlines proposed rate changes across participating individual market carriers.

Table 1

Proposed 2027 Individual Market Rate Requests by Carrier and Rating Areas

Carrier Name	Requested Rate Change (%)	Geographic Rating Areas
Ambetter Health of Pennsylvania	40.90%	1–9
Keystone Health Plan Central	33.83%	6, 7, 9
Highmark Benefits Group	21.40%	3, 8
Partners Insurance Company	20.51%	3, 6, 8
QCC Insurance Company	19.88%	8
Capital Advantage Assurance Company	18.32%	6, 7, 9
Highmark Inc.	16.16%	1, 2, 4, 5, 6, 7, 9
Health Partners Plans	15.90%	3, 6, 8
Oscar Health Plan of Pennsylvania	15.39%	3, 6, 7, 9
Keystone Health Plan East	14.68%	8
Geisinger Quality Options	12.95%	2, 3, 5, 6, 7, 9
Geisinger Health Plan	10.46%	2, 3, 5, 6, 7, 9

Note. Adapted from PID (2026). Additional filings noted in PID primary announcements include UPMC Health Network, Inc. (+12.03%) and UPMC Health Plan, Inc. (+15.82%). Spread in requested increases across supplied carriers equals $40.90\% - 10.46\% = 30.44\%$ percentage points.

Deconstructing Ambetter's Rate Filing Mechanics

The highest single individual market rate request—submitted by Ambetter Health of Pennsylvania at 40.90%—is driven by a structural plan tier exit rather than an across-the-board rate hike on existing benefit packages. PID's statutory review indicates that Ambetter's filing combines two distinct mathematical components:

1. **Silver-to-Silver Base Rate Increase:** A base premium adjustment of 23.44% applied to silver metal tier plans.
2. **Bronze-to-Silver Plan Mapping Differential:** A 17.46% premium differential resulting from Ambetter's withdrawal from the bronze metal tier, which triggers automatic administrative mapping of existing bronze policyholders into silver tier plans (PID, 2026).

Consequently, while the reported top-line rate increase is 40.90%, existing silver enrollees face a 23.44% increase, whereas bronze policyholders mapped into higher-actuarial-value silver plans absorb the combined cost effect.

Small-Group Market Rate Filings

Small-employer health insurance markets demonstrated less extreme top-line variance than the individual market, though proposed increases remain well above general cost inflation. The enrollment-weighted small-group average requested increase was 11.5% (PID, 2026).

Table 2

Proposed 2027 Small-Group Market Rate Requests by Carrier

Carrier Name	Requested Rate Change (%)	Rating Areas / Notes
Highmark Care Benefits	19.95%	Rating Area 3
Highmark Benefits Group	16.99%	Rating Areas 6, 7, 9
Highmark Inc.	15.57%	Rating Areas 1, 2, 4, 5, 6, 7, 9
Highmark Coverage Advantage	14.63%	Rating Areas 1, 2, 4, 5, 6
Highmark Senior Health Company	14.32%	Rating Area 8
Geisinger Quality Options	4.05%	Rating Areas 2, 3, 5, 6, 7, 9
UnitedHealthcare of Pennsylvania	Not Stated	Includes Gold/Silver-to-Bronze plan mapping

Note. Adapted from PID (2026). Additional small-group rate filings cited in PID records include Keystone Health Plan Central (+20.93%), Capital Advantage Assurance Company (+16.11%), Independence Assurance Company (+8.95%), and Geisinger Health Plan (+3.74%). The spread between extreme values is $19.95\% - 4.05\% = 15.90\%$ percentage points.

Pennie Marketplace Attrition and Public Health Equity Implications

Longitudinal Enrollment Contraction

Following the expiration of enhanced federal premium subsidies under the American Rescue Plan Act (ARPA) and Inflation Reduction Act (IRA) on December 31, 2025, Pennie observed systematic, month-over-month enrollment decay throughout 2026.

Table 3

Active Pennie Marketplace Enrollment Tracking (Plan Year 2026)

Measurement Date	Active Enrollees	Net Change from Feb 1	Percent Change (%)
February 1, 2026	486,000	—	—
March 24, 2026	471,442	-14,558	-3.00%
May 1, 2026	452,000	-34,000	-7.00%
July 1, 2026	431,270	-54,730	-11.26%
August 1, 2026	422,869	-63,131	-12.99%

Note. Data compiled from Pennie administrative dashboards (2026). Total attrition from Feb 1 to Aug 1 equals \Delta = 63,131 individuals, representing a relative contraction of approximately 13.0\%.

As of August 2026, Pennie reported three distinct loss metrics:

- **85,000 Open Enrollment Non-Effectuations:** Individuals who selected a plan during open enrollment but failed to remit initial binder payments.
- **108,000 Post-Enrollment Terminations:** Active consumers who dropped coverage mid-year following the initial close of open enrollment.
- **>191,000 Cumulative Cancellations:** Aggregate terminations including both non-effectuations and mid-year plan drops.

Public Health and Population Equity Analysis

The convergence of double-digit rate hikes (17.1% individual average) and post-subsidy price increases presents severe public health risks across Pennsylvania:

1. **Exacerbation of Health Disparities and Preventable Mortality:** The loss of 63,000+ active enrollees disproportionately affects lower-middle-income individuals who fall just above Medicaid eligibility thresholds. Uninsured individuals frequently forego routine care, leading to delayed diagnoses of cardiovascular disease, diabetes, and oncological conditions.
2. **Coverage Churn and Treatment Interruption:** Forced plan reassignment—such as Ambetter’s transition of bronze enrollees to silver plans—disrupts provider network continuity. Patients undergoing active treatments (e.g., chemotherapy, mental health therapy, substance use recovery) face catastrophic administrative friction, re-authorization delays, or cost-sharing spikes.
3. **Underinsurance and Financial Vulnerability:** As silver premiums rise by 15\% \text{ to } 33\%, low-income enrollees may downgrade to high-deductible plans or drop coverage entirely. Increased out-of-pocket exposure drives medical debt, prescription non-adherence, and emergency department overutilization.

Open Enrollment 2027 Regulatory Timeline and Legal Framework

The administrative execution of Plan Year 2027 open enrollment was heavily shaped by federal rule-making and federal court intervention.

[Spring 2026 2026 – Jan 15, 2027]	[July 16, 2026]	[Nov 1,
Pennie Board shortened restored 15-week OE window	U.S. District Court stayed	Pennie
OE window to 9 weeks	CMS 9-week rule limit	

Coverage effective Jan 1 / Feb 1

(Oct 15 – Dec 15, 2026)

(City of Columbus v. Kennedy)

(Confirmed by CMS July 31)

In early 2026, Pennie's Board of Directors aligned state policy with the 2027 Notice of Benefit and Payment Parameters, shortening open enrollment to 9 weeks (October 15–December 15, 2026). However, on July 16, 2026, the U.S. District Court for the District of Maryland granted a nationwide stay in *City of Columbus v. Kennedy* (No. 26-2215). CMS subsequently issued formal administrative guidance on July 31, 2026, confirming that state exchanges could maintain traditional enrollment windows (CMS, 2026; PORH, 2026).

Consequently, Pennie reinstated the traditional 15-week schedule:

- **Open Enrollment Window:** November 1, 2026 through January 15, 2027.
- **Coverage Start Dates:** January 1, 2027 (for plan selections completed by December deadline); February 1, 2027 (for selections completed between mid-December and January 15).

Policy Recommendations for Public Health Action

To mitigate rate shock and maintain market stability, state policymakers and public health administrators should pursue four key interventions:

1. **Rigorous Insurance Department Rate Review:** PID should utilize its full statutory authority to scrutinize carrier administrative costs and medical loss ratios, rejecting unjustified increases that exacerbate plan drop-outs.
2. **Targeted Re-enrollment and Navigational Support:** State funding should prioritize outreach to the 85,000 non-effectuated consumers and former Ambetter bronze enrollees, ensuring smooth transition into alternative affordable plans.
3. **State-Level Premium Subsidies:** Pennsylvania should evaluate state-funded premium tax credits to fill the coverage gap left by expiring federal ARPA/IRA subsidies.
4. **Monitoring Network Adequacy:** PID must ensure that cross-tier plan mapping (e.g., bronze to silver) does not strip vulnerable enrollees of access to essential specialty care networks.

References

City of Columbus v. Kennedy (Columbus II), No. 26-2215 (D. Md. July 16, 2026).

Centers for Medicare & Medicaid Services. (2026, July 31). *Guidance acknowledging the stay of the 2027 Notice of Benefit and Payment Parameters*. U.S. Department of Health and Human Services.

Pennsylvania Insurance Department. (2026, July 23). *Shapiro administration receives proposed 2027 health insurance rates; plans to carefully examine companies' proposals and reject excessive, inadequate, or unfairly discriminatory increases* [Press release]. Commonwealth of Pennsylvania.

Pennsylvania Office of Rural Health. (2026, July 25). *PA health insurance exchange 2027 open enrollment period returns to November 1, 2026–January 15, 2027*. Pennsylvania State University.

WVIA. (2026, September 6). *Pa. health insurers propose 2027 rate hikes as Pennsylvanians*

grapple with rising costs. WVIA News.

Appendix A: Chronological Timeline of Rate Filings, Litigation, and Regulatory Milestones

Table A1

Exhaustive Administrative, Legal, and Market Event Log (2025–2027)

Date	Event / Action	Market / Subject	Detailed Description	Source	Status / Significance
2025-12-31	Federal Subsidy Expiration	Federal Subsidy Policy	Enhanced federal premium tax credits expired, increasing net premiums and sparking cancellations.	PID (2026); WVIA (2026)	Historical Policy Context
2026-01-01	Subsidy Expiration Impact	Individual Market	Consumers experienced full net premium increases for Plan Year 2026 coverage.	WVIA (2026)	Historical Policy Context
2026-01-31	Close of OE 2026	Pennie	Open Enrollment 2026 concluded; serves as baseline for post-enrollment attrition tracking.	Pennie (2026)	Baseline Reference Date
2026-02-01	Enrollment Count	Pennie	Active Pennie enrollment reported at peak level of ~486,000 enrollees.	Pennie (2026)	Attrition Baseline Start
2026-03-01	Pennie Board Vote	Federal/State Rules	Pennie board voted to shorten PY 2027 OE to Oct 15–Dec 15 per federal 9-week rule.	PORH (2026)	Initial 2027 Schedule

Date	Event / Action	Market / Subject	Detailed Description	Source	Status / Significance
2026-03-24	Enrollment Count	Pennie	Active Pennie enrollment measured at 471,442 enrollees.	Pennie (2026)	Attrition Tracking
2026-05-01	Enrollment Count	Pennie	Active Pennie enrollment dropped to ~452,000 enrollees.	Pennie (2026)	Attrition Tracking
2026-05-15	Rate Filing Deadline	Regulatory	Insurers submitted proposed 2027 rate filings to Pennsylvania Insurance Dept.	PID (2026)	Regulatory Milestone
2026-07-01	Enrollment Count	Pennie	Active Pennie enrollment recorded at 431,270 enrollees.	Pennie (2026)	Attrition Tracking
2026-07-16	District Court Stay	Litigation	U.S. District Court granted nationwide stay halting federal 9-week OE restriction.	<i>City of Columbus v. Kennedy</i> (2026)	Major Legal Ruling
2026-07-23	Proposed Rates Release	Individual / Small-Group	PID published proposed rate increases (17.1% individual avg, 11.5% small group avg).	PID (2026)	Official Rate Release
2026-07-23	Public Comment Opens	Regulatory	PID formally opened public comment period on proposed filings through August 22.	PID (2026)	Public Participation
2026-07-23	Ambetter Decomposition	Individual Market	PID explained Ambetter 40.90% request	PID (2026)	Plan Mapping Analysis

Date	Event / Action	Market / Subject	Detailed Description	Source	Status / Significance
			(23.44% base + 17.46% mapping differential).		
2026-07-25	Public Schedule Guidance	Pennie / OE 2027	PORH reported Pennie's restoration of 15-week OE (Nov 1, 2026–Jan 15, 2027).	PORH (2026)	Schedule Restoration
2026-07-31	CMS Federal Guidance	Federal Regulatory	CMS published guidance confirming nationwide stay and 15-week OE restoration.	CMS (2026)	Federal Administrative Action
2026-08-01	Enrollment Count	Pennie	Active Pennie enrollment measured at 422,869 enrollees (13.0% decline from Feb 1).	Pennie (2026)	Attrition Tracking
2026-08-01	Post-OE Loss Metric	Pennie	~108,000 consumers reported dropping coverage mid-year post-OE close.	Pennie (2026)	Attrition Metric
2026-08-01	Cumulative Cancellations	Pennie	>191,000 cumulative cancellations reported including OE terminations.	Pennie (2026)	Aggregate Loss Metric
2026-08-01	Non-Effectuations	Pennie	~85,000 consumers classified separately as OE non-effectuations (unpaid binders).	Pennie (2026)	Payment Deficit Metric

Date	Event / Action	Market / Subject	Detailed Description	Source	Status / Significance
2026-08-10	Dashboard Update	Pennie	Pennie updated affordability dashboard with revised financial burden metrics.	Pennie (2026)	Data Update
2026-08-10	Net Premium Report	Pennie / Cost Burden	Average net monthly premium payment reported to have increased by ~102%.	WVIA (2026)	Cost Burden Analysis
2026-08-22	Public Comment Deadline	Regulatory	Formal closing date for public comments on proposed 2027 health rate filings.	PID (2026)	Regulatory Deadline
2026-09-07	Current Status Date	Regulatory Review	Filings remain under active review by Insurance Department actuaries.	PID (2026)	Status Benchmark
2026-09-30	Finalization Target	Regulatory Target	Expected timeframe for PID to complete rate reviews and announce final rates.	WVIA (2026)	Expected Target
2026-11-01	OE 2027 Begins	Marketplace	Restored 15-week Open Enrollment period for Plan Year 2027 formally begins.	PORH (2026); CMS (2026)	Enrollment Milestone
2026-12-15	Initial Plan Target Date	Marketplace	Original shortened OE deadline; now deadline for coverage effective Jan 1.	PORH (2026)	Coverage Effective Cutoff
2027-01-01	Coverage	Marketplace	Plan Year 2027	PORH (2026)	Effective Date

Date	Event / Action	Market / Subject	Detailed Description	Source	Status / Significance
	Effective Date		coverage begins for selections submitted by Dec 15 deadline.		
2027-01-15	OE 2027 Closes	Marketplace	Final closing date for Plan Year 2027 individual marketplace open enrollment.	PORH (2026); CMS (2026)	Enrollment Deadline
2027-02-01	Second Effective Date	Marketplace	Coverage effective date for selections completed between Dec 16 and Jan 15.	PORH (2026)	Effective Date
2027-Fall	Final Approved Rates	Regulatory	PID publishes final approved 2027 rate adjustments following administrative review.	PID (2026)	Final Regulatory Output